

Patient details

Full
name

DoB

Unit
number

(use sticker if available)

Emergency Decision Unit Pathway

Head injury

Date DD/MM/YY

Has lived in UK >1 year?

☐ Yes ☐ No

Inclusion criteria

ED patient awaiting CT head and/or requiring observation in hospital (see reverse for indications) following a head injury.

NB: 'Head injury' is defined as any trauma to the head leading to at least one of the following:

- witnessed loss of consciousness (LOC)
- definite amnesia
- witnessed disorientation
- GCS < 15
- head injury cannot be excluded because patient intoxicated

Exclusion criteria

- Patient with 'normal' CT brain who need geriatric expertise (admit to SSA or RAU as indicated)
- GCS < 13 prior to CT brain (keep in ER; involve ITU if uncooperative)
- GCS < 9, or 9 - 12 and uncooperative
 - admit to ITU if active, non-surgical treatment
 - use 'EDU pathway - TLC' if no active treatment
- Patients requiring transfer to QMC neurosurgery (keep in ER)
- Patients requiring admission under a specialist team for other injuries or illnesses

Notes to doctor completing this form

Please document on this page

- how long patient is to be observed, and indication
- what results or tests are still pending (if any)
- what treatments are required (if any)
- any advice obtained during neurosurgical telephone consultation

EDU plan

- Regular neurological observations (see reverse for details)
- Refer for ENT assessment if base of skull fracture and
 - Haemotympanon
 - CSF otorrhoea
 - Hearing with or without balance problem
- Review and arrange / repeat CT urgently if
 - Falling GCS
 - First post-traumatic seizure on EDU
 - New focal neurological deficit on EDU
- Repeat CT brain if GCS still < 15 at next day ward round
- Transfer 'CT positive' patients who require geriatric expertise to SSA or RAU as indicated after 24h if remaining stable
- Consultant will advise about need for admission to LGH brain injury unit (see reverse for practicalities) if condition not improved enough to permit safe discharge after 48h
- Consultant will advise about need for neurology OPD follow-up if CT showed traumatic parenchymal lesion or intracranial haematoma
- Give patient / carer head injury advice leaflet on discharge

Planned & agreed by

Referring consultant

Doctor in charge

Nurse in charge

Print names

Signatures

NB: A drug chart must be written before pathway can be signed off

Indications for observation in hospital following a head injury

Indication...

... and how long to observe

- | | |
|--|---|
| <ul style="list-style-type: none">• Abnormal CT brain• Absence of responsible adult at home• GCS still < 15• Post-traumatic seizure• Focal neurological deficit• Coagulopathy, taking anticoagulants or platelets < 100 | <ul style="list-style-type: none">• until next consultant ward round• overnight• 8h after GCS has returned to 15• until next consultant ward round• until next consultant ward round• keep for 24h |
|--|---|

Also consider if no CT was performed:

- | | |
|---|---|
| <ul style="list-style-type: none">• Amnesia before impact > 30min• Dangerous mechanism<ul style="list-style-type: none">• pedestrian hit by motor vehicle• ejected from motor vehicle• fall > 3 feet or five steps | <ul style="list-style-type: none">• 8h• 8h |
|---|---|

NB: Always keep overnight if discharge after 8h impractical

How to perform neurological observations - guidance for nursing staff

What to check...

... and how often

- | | |
|---|---|
| <ul style="list-style-type: none">• GCS• Pupil size and reactivity to light• Limb movements• Temperature• Pulse• Respiratory rate• SaO₂• BP | <ul style="list-style-type: none">• Half-hourly until GCS 15• Half-hourly for next 2 hours• 1 hourly for next 4 hours• 2 hourly thereafter• Half-hourly again if deteriorates |
|---|---|

Referrals to Brain Injury Unit (BIU) at LGH - guidance for medical staff

- Avoid referrals that would result in the patient arriving at LGH after 16:00 (discuss option of keeping patient on EDU for a further night with senior)
- Inform bed bureau (☎ 4858; state 'Head injury patient for BIU' - bed bureau staff will know what to do even if there is no BIU bed available) as early in the day as possible
- Inform the BIU team by calling either Dr. Peter Critchley's secretary (☎ 4496) or BIU directly (☎ 4628 or 4629)